Phenobarbital - Phenobarbitone

****This monograph is under review****

Neonatal

FORM Ampoule containing 60mg in1ml

INDICATION

1. Control of neonatal seizures.
2. Sedation in Neonatal abstinence syndrome.

DOSE RANGE

Indication 1. Control of neonatal seizures.

AGE	DOSE	FREQUENCY	ROUTE
Neonate Loading	20mg/kg/dose	Single *	IV
Maintenance**	2.5 - 5mg/kg/dose	1 dose daily	IV

***LOADING DOSE give 20mg/kg (given over 20minutes) if baby fitting. If fitting persists, give another 10mg/kg (given over 10 minutes) after 20 minutes. If fitting persists, a final 10mg/kg (given over 10 minutes) should be given after a further 20 minutes. Maximum total loading dose 40mg/kg. Care required as apnoea may be a problem.**

**** see Neonatal seizures policy. Maintenance phenobarbital only to be used on direction of a Consultant Paediatrician. Maintenance dose commences 24 hours after loading dose.**

Injection to be diluted before administration – see below

Indication 2. Sedation in Neonatal abstinence syndrome.

AGE	DOSE	FREQUENCY	ROUTE
Neonate Loading	15mg/kg/dose	Single	IV
Maintenance commenced 24 hours after loading dose	8mg/kg/dose	1 dose daily	IV

RECONSTITUTION Already in solution – must be diluted prior to use

DILUTION

Phenobarbital injection (60mg in 1ml) 1ml
Water for injection up to 6ml total

Gives 10mg in 1ml solution for IV use. Use required volume.

For loading dose for babies >2.3kg use:

Phenobarbital injection(60mg in 1ml) 2ml
Water for injection up to 12ml total

Gives 10mg in 1ml solution for IV use. Use required volume.

METHOD OF ADMINISTRATION	For IV administration to be given slowly at a maximum rate of 1mg/kg/minute.

COMPATIBILITY

Solution compatibility	sodium chloride 0.45%, sodium chloride 0.9%, glucose 5%, glucose 10%
Solution incompatibility	No information
IV Line compatibility	Aciclovir, caffeine citrate,calcium gluconate,dopamine,fluconazole, furosemide,meropenem,metronidazole,milrinone,potassium chloride sodium bicarbonate,vancomycin,vecuronium
IV Line incompatibility	adrenaline,cefotaxime,dobutamine,esmolol, midazolam,noradrenaline,phenytoin,pyridoxine

THIS LIST IS NOT EXHAUSTIVE PLEASE CONTACT PHARMACY FOR FURTHER INFORMATION ON COMPATIBILITY WITH ANY MEDICINES NOT INCLUDED

CAUTIONS, CONTRA-INDICATIONS AND SIDE EFFECTS

- See Summary of Product Characteristics and most recent edition of BNF for Children

FURTHER INFORMATION

Blood Levels-

- Approximate time to steady state is 10-14 days.
- Therapeutic range 15 to 40mg/Litre.
- Take trough sample, immediately prior to next dose.

- High extravasation risk.
- Phenobarbital reduces blood levels of Carbamazepine. The effect of Phenobarbital on Phenytoin levels is unpredictable therefore blood levels should be checked if this drug combination is initiated.
- As a general CNS depressant, Phenobarbital may produce respiratory depression requiring assisted ventilation.
- Contains propylene glycol 90%

PH 10 - 11

LICENSED STATUS Not licensed by IV route in children, nor for neonatal abstinence syndrome.

APPLICABLE POLICIES West of Scotland Neonatal Guidelines

Consult local policy if applicable

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Administer reconstituted solutions immediately.
All vials, ampoules and infusion bags are for single use only unless otherwise stated.

Dose may vary depending on indication, age, renal function, hepatic function, and concomitant medications.
This monograph should be used in conjunction with the terms of reference document prepared by the West of Scotland Pharmacist Network. Information is correct at the time of publication and as per local practice agreement.
For further advice please contact your clinical pharmacist or pharmacy department